STRATEGY RESEARCH

Centen

Competitive landscape and product recommendations for a preventive-health behaviour coach

The opportunity

Turn a long-term healthspan ambition into the smallest health-promoting action that fits the day a person actually has.

Competitive focus: Google Health, Don't Die and Sweetch, with adjacent lessons from Oura, WHOOP, Omada, Lark, Noom, Fabulous, Evidation, Function and Samsung Health.

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Executive view

Centen should not try to outdo Google at being an all-purpose AI health companion, nor Sweetch at enterprise chronic-care breadth. Its credible wedge is narrower and more coherent: a private, explainable engine that chooses the smallest health-promoting action likely to happen in a particular moment - and treats non-action as learning, never as user failure.

> **The strategic proposition**
>
> **Centen helps you turn the years you want to feel well into small actions that fit today.** It balances movement, nourishment, recovery, mental wellbeing and connection - without turning health into a score to chase.

What makes this credible

- **Feasibility before ambition.** Receptivity determines whether and when to ask; barriers determine the dose.
- **Success-only progress.** A completed two-minute action reinforces agency; misses are model evidence, not feedback.
- **Privacy as product value.** On-device inference, optional data sources, and understandable decision receipts build trust.
- **Restraint is a feature.** The null action is always eligible; Centen optimises health behaviour, not notifications or app opens.

Competitive map

Product	What it does well	Centen opening
Google Health / Coach	Broad connected data, polished conversational AI, adaptive plans, flexible logging.	Be transparent at the action level; work without a device or premium subscription; optimise whether, when and how much to ask.
Don't Die	Identity, community, biomarker interest, broad wearable integrations and a motivating scorecard.	Serve real, variable-capacity days without competition, streak pressure or an optimiser-only worldview.
Sweetch	The nearest model: JITAI, context-aware micro-goals, adaptive interventions, clinical and enterprise readiness.	Offer a calmer consumer product with visible barrier reasoning, local inference and causal decision evidence.
Oura / WHOOP	Recovery and healthspan framing make longitudinal biometrics understandable.	Turn readiness into a feasible next action for people with or without premium hardware.
Omada / Lark	Clinical programmes, coaching and care pathways for chronic conditions.	Stay useful before diagnosis or coverage, while preserving a careful referral boundary.

The priority competitors

Google Health

Google Health Coach sets a high bar for personalised, adaptive coaching across fitness, sleep, mindfulness and meals. It can adjust plans using readiness, progress and weather, and supports conversational data capture. Its public experience is principally an adaptive plan plus adviser. Centen should borrow the daily synthesis and easy plan editing, while making its own decision process legible: why this action, why this dose, and why not a larger one.

The opening: Google currently ties full coaching to Premium and compatible Fitbit or Pixel Watch hardware. Centen's P0/P1 modes can remain genuinely useful without a wearable, and on-device inference can be a tangible trust advantage.

Don't Die

Don't Die is effective at making longevity visible and socially meaningful. It combines a daily score, progress sharing, local communities, wearables and biomarker/biological-age interest. The counter-position is equally clear: Centen should never make a person feel that a difficult day has lowered their worth or erased their progress.

Borrow the emotional clarity of a long-horizon mission and optional belonging. Do not borrow comparison, ranking, score-chasing or the high-commitment protocol framing.

Sweetch

Sweetch is the closest strategic comparator. It describes a clinically validated JITAI platform that senses smartphone and connected-device data, dynamically optimises plans and engages people with contextual micro-goals. It also demonstrates the later enterprise opportunity - care programmes, connected devices, dashboards and EHR integration.

Centen's advantage is focus. Make the engine's reasoning visible, use the least data that improves the next action, keep inference local, and begin with everyday capability rather than chronic-condition management.

Centen as a health portfolio

The first walking product remains the right validation wedge. The fuller Centen experience should share one behaviour engine across several action portfolios - not present five disconnected trackers.

Portfolio	First action classes	Decision and boundary
Movement	Walking, cardio, strength, mobility and movement breaks.	Select timing, dose and fallback; maintain distinct cardio/strength ladders; never prescribe rehabilitation.
Nourishment	Meal planning, prepare a default meal, add-a-food and hydration cues.	Cue a smallest feasible preparation or choice; avoid calorie targets and moralised food labels.
Recovery	Wind-down, daylight, sleep preparation and active rest.	Prefer recovery over progressive training when appropriate; no sleep-disorder diagnosis or treatment.
Mental wellbeing	Breathing, reflection, screen boundary, enjoyable activity and support-resource prompt.	Reduce demand under stress; never simulate therapy or exploit distress to increase engagement.
Social connection	Message or call someone, plan a shared walk, accept an invitation.	Find low-friction connection; never inspect contacts or messages without explicit purpose-specific consent.

One cross-domain governor

- One primary health action at a time.
- A user-defined weekly focus with a small maintenance minimum for other domains.
- One burden budget across all domains - no pile-up of individually reasonable nudges.
- Recovery and safety take precedence over progressive training.
- Pair actions only when they reduce friction, for example a social walk; never manufacture engagement.

The Centen product experience

Moment	What Centen should do
Morning	Offer one realistic theme, not a programme: ‘Energy looks lower today. Your foundation action is ten minutes outside after lunch. Strength can wait.’
In the moment	Offer a minimum viable action: ‘Your meeting ended early. Five minutes of mobility or a short walk?’
Evening	Frame recovery as productive: ‘You already built your foundation today. Let’s make tomorrow easier.’
Weekly	Invite choice: keep an anchor, try one strength session, or stay with walking. The user chooses the focus.
After disruption	Offer re-entry without debt: ‘Things changed. Start again with the smallest version that fits.’

Do not lead with a health-age score

WHOOP demonstrates that healthspan and pace-of-aging are compelling consumer concepts. It also explicitly positions its measure as a wellness estimate without a clinical validation benchmark. Centen should use healthspan as a long-horizon direction, not as a daily judgement or notification trigger.

Use a two-layer model instead: a visible, non-decreasing Foundation for repeated actions now; and a monthly or quarterly Healthspan Habits reflection that names what has become reliable and lets the person select the next focus.

Engine extensions before breadth

- **Action-class schema:** add domain, prerequisites, contraindications, effort/recovery load, equipment/location needs, verification method and a domain-specific progression ladder.
- **Cross-domain trade-offs:** model substitution and complementarity explicitly. A late hard workout may compete with sleep; a social walk may support movement and connection.
- **User priorities as a constraint:** Centen may recommend but must not silently change a person's goal or focus allocation.
- **Evidence hierarchy:** distinguish objective confirmation, lightweight self-report and inferred context. Weak inference never overrides safety or a user correction.
- **Domain safety policies:** action libraries need reviewed caps and exclusion rules; an unrestricted chat system must not improvise health advice.
- **Portfolio-level experimentation:** randomise only inside safe, understandable alternatives and log the whole candidate set.

Recommended sequence

Stage	Product focus	Why it comes now
1. Foundations	Walking plus recovery-aware dose selection, planning touchpoint, foundation and re-entry.	Prove that the smallest feasible action happens more often and feels better.
2. Movement complete	Strength ladder, mobility and cardio variety with a shared burden budget.	Extends a coherent portfolio and tests safe progression beyond walking.
3. Recovery + nourishment	Sleep-prep/active-rest routines and meal-prep/add-a-food actions.	High value and lower clinical risk when kept behavioural and non-prescriptive.
4. Wellbeing + connection	Stress-sensitive demand reduction, reviewed micro-actions, optional social activity planning.	Requires mature consent, content and escalation design.
5. Data-rich layer	Wearables, labs/biomarker import, healthspan reflection and partner integrations.	Should enrich rather than become required for the core coach.
6. Supplements	At most a neutral, user-controlled record or clinician-set reminder.	Personalised recommendations, dosing and interactions require clinical governance and regulatory work.

Three traps to avoid

- **The dashboard trap:** more insight cards do not create behaviour change. Every insight must earn its place by changing the next feasible action.
- **The score trap:** a longevity number can motivate but can also make wellbeing feel like a test. Keep long-range indicators optional and reflective.
- **The chatbot trap:** chat is an interface, not a behaviour model. Use it to articulate barriers and explain choices; BUE remains accountable for timing, dose and safety.

Pilot questions

1. Do users experience a smaller step as smart adaptation rather than lowered expectation?

2. Does an editable ‘what Centen believes today’ view increase trust and correction quality?

3. How much added value does a daily check-in offer compared with app-only use?

4. Is a deliberate no-proposal understood as respectful restraint?

5. Can success-only progress sustain motivation for 12 weeks without scores or streaks?

6. Which barrier-linked tactics add value beyond adaptive timing and dose?

Sources

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